

What changed on July 2, and why it is on your desk

A briefing for Alberta physicians and nurse practitioners completing the DIA Medical Report (DS2444B). September 2026.

What happened

On 2 July 2026, AISH recipients were moved to the Alberta Disability Assistance Program (ADAP). The move itself cannot be appealed. A patient who wants to return to AISH must make a new application, which requires a medical report on form DS2444B.

ADAP pays \$1,740 a month, plus a \$200 transition benefit that ends 31 December 2027, and carries employment expectations that AISH does not. Many of these patients were assessed as severely handicapped years ago and are being asked to establish it again, often with conditions that have progressed since.

You are paid for this report

Form EMP11587 authorises payment for the examination and for completion of the report, as two separate fees, invoiced by you directly to the government. It must be arranged **before** the report is completed, not after. Your patient requests it and can have it sent to your office.

A nurse practitioner may complete the DS2444B. It is not restricted to physicians.

The test the report is weighed against

A severe and permanent impairment that substantially limits the ability to earn a livelihood. Every field below is read against that test.

- **Rate to the clinical picture.** Q11–Q40 and Q41–Q68 run 0 to 3. Under-rating to appear conservative weakens the file. Complete the describe box on every 2 and 3.
- **Do not average variability.** For episodic conditions write the range, not the mean: “3 on flare days, 1 between,” rather than a blended 2. A good day is not the measure; the pattern is.
- **Cumulative effect is asked for twice.** Q40 and Q68 both ask for the cumulative level of impairment. List comorbidities at Q5 even where individually sub-severe.
- **Accommodation language works against the patient.** It reads as capable with support, which is the ADAP finding rather than the AISH one. Where clinically true, state that no accommodation makes competitive employment reliable or sustainable.
- **Prognosis at Q75 is determinative.** Where clinically supported, Permanent. Undetermined tends to default the file to the lower program. For episodic conditions, quantify frequency, duration and severity.
- **Explain any non-adherence.** An unexplained “non-compliant” can sink a strong file. Cost, side effects, access, and executive dysfunction affecting medication management belong on the record.
- **Netcare is not the full record.** It holds labs, imaging, medications and hospital records, not community clinic notes. Draw also on the chart, pharmacy record and specialist correspondence.

Accuracy is the strongest position

An overstated report can be challenged, and it can cost a patient a case they truly have. Nothing here asks you to record a finding you did not make. The purpose is to put your real findings into the language the determination weighs, and no further.

The full question-by-question guide to the DS2444B, and a fillable copy of the form that opens on any phone, are free at albertadisabilitysystembreakdown.netlify.app. The official form is at open.alberta.ca. The Alberta Disability System Breakdown is a free community campaign, not a government body, and receives no government funding. General information, not legal or medical advice. Free to share and print.